

Patient 16**Report A**

654412926/06/2016 CT Acute Stroke

For Thrombolysis? No

Report:

Time 1st called: 18:14

Time plain CT available on PACS: 18:20

Time of verbal report to stroke registrar: 18:25

Indication: Left upper limb paresis.? Stroke.? Todds.

CT HEAD

Haemorrhage: No

Intravascular thrombus: No

Acute infarct: No Other findings and comments: Generalised involutional change which is slightly more prominent in the medial temporal lobes; associated ex vacuo dilatation of ventricles. Low attenuation in the left anterior temporal lobe is suggestive of previous infarct. There is background periventricular and white matter low attenuation consistent with mild chronic small vessel disease.

CT ANGIOGRAM

Window settings: (intraluminal HU=300; W=600; L=200)

(ICA Stenoses NASCET method, measurements on axial 1mm sections) Aortic arch and subclavian arteries: Normal anatomical configuration of the aortic arch and branches.

RCCA/ICA: Mixed non-stenosing atheroma

LCCA/ICA: Mixed non-stenosing atheroma

RVA. Mixed atheroma within the right VA, with calcific stenosis in the V4 intradural segment with small calibre vessel demonstrated distally, Distal opacification of V4 suggests patency

L VA: Non-stenosing atheroma. Dominant. intracranial Arteries: The circle of Willis and proximal MCA, ACA and PCA branches are patent bilaterally with no intracranial arterial aneurysms, intravascular thrombus or vessel occlusion demonstrated. The right M2 is small in calibre at the branching point off the right M1, but demonstrates distal opacification and suggests patency. The left A1 is not well visualised and may be hypoplastic. The basilar artery is patent.

Other findings and comments: The lung apices are clear. There is mucosal thickening in the maxillary antra and right frontal sinus. Extensive degenerative change is demonstrated in the C spine, however no acute destructive focal bony lesion demonstrated.

Conclusion: No acute intracranial infarct demonstrated.

Stroke Reg informed of findings at time of scanning.

Report B

654415627/06/2016 MR Head

6544156 27/06/2016 MR MRA Clinical indications for MRI - Admitted with left upper limb weakness. CT showed possible minor subarachnoid haemorrhage in the posterior aspect of the right temporal lobe, ? stroke

Findings: There is restricted diffusion seen in the right peri-Rolandic region (pre and post central gyrus) and a focus of restricted diffusion is seen in the posterior left frontal lobe in keeping with acute/subacute infarct. The two different territories suggest a central thromboembolic source. Moderate chronic microangiopathic disease demonstrated. No acute haemorrhage. Generalised age-related neuroparenchymal volume loss with a predilection for the temporal lobe. There is resulting ventriculomegaly. No proximal intravascular thrombus or intracranial aneurysm demonstrated.